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4 May 2026

Richard E. Besser, MD

President and CEO

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Dear Dr. Besser:

Thank you for attending my oral presentation regarding veteran homelessness in Newark, New Jersey and for your consideration of the Newark Veterans Reintegration Program (NVRP). This proposal documents the urgent crisis of veteran homelessness in the city, a population facing various barriers of untreated mental illness, substance abuse, and institutional distrust that cause traditional intervention approaches to be widely ineffective. Drawing on peer-reviewed and government data as well as established local initiatives, the NVRP proposes a twelve-month, community-driven program built around two evidence-based models: peer support specialist outreach and in-person harm reduction delivery. The program will be implemented in partnership with Bridges Outreach, New Jersey's designated service provider for Essex and Union counties.

The Robert Wood Johnson Foundation's mission to advance health equity and support vulnerable community members makes it the ideal partner for this program. A \$503,000 investment in the Newark Veterans Reintegration Program would directly improve the lives of Newark's homeless veteran population and would generate a replicable and evidence-backed model that could scale across the state.

I welcome any questions or additional discussion and look forward to your response. I can be contacted at jake.adams@rutgers.edu or (908) 839-6606.

Sincerely,

Jake Adams

Jake Adams

Addressing Veteran Homelessness in Newark, New Jersey



Submitted by:

Jake Adams

Submitted to:

Richard E. Besser

President and CEO of Robert Wood Johnson Foundation

50 College Road East, Princeton, NJ 08540

Abstract

Veteran homelessness in Newark, New Jersey represents a public health crisis in urgent need of intervention, with an estimated 174 or more veterans experiencing homelessness in Essex County and hundreds more at risk. This proposal examines this crisis and presents a research-based plan of action to address it. Drawing from peer-reviewed articles, government publications, and local initiative reports, the proposal documents the health consequences among Newark's homeless veteran population, including heightened rates of PTSD, traumatic brain injury, substance abuse, and suicide. The effectiveness of existing intervention strategies will be analyzed, identifying the most evidence-backed methodologies that will be utilized to address Newark's crisis while documenting the consistent failures of various other programs. Based on these findings, the proposal presents the Newark Veterans Reintegration Program, a twelve-month pilot initiative enacted in partnership with Bridges Outreach involving peer outreach, harm reduction services, benefits navigation, and community integration programming. A detailed budget, evaluation plan, and discussion of anticipated outcomes are included. Approximately \$503,000 of funding will be requested from the Robert Wood Johnson Foundation to implement this program.

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Introduction

The Detrimental Effects of Homelessness on Veterans in Newark

The U.S. Department of Veterans Affairs (2019) documents that a large but unquantifiable portion of the homeless veteran population suffers from chronic depression, post-traumatic stress disorder, and traumatic brain injuries. These are conditions that often develop during active service, yet go undiagnosed until years of homelessness have already passed, making it significantly more difficult to treat them on top of already being the biggest risk factors driving veteran homelessness. These conditions are what primarily drive the cycle of chronic homelessness, as mental illness leads to housing instability which ultimately worsens said mental illness (U.S. Department of Veterans Affairs, 2019). Nearly half or more homeless veterans are diagnosed with mental disorders, a rate more than double that of their housed counterparts (NCHV, 2024).

Newark, New Jersey is one of the most densely populated and diverse cities in the United States, thus containing a large number of homeless veterans and an environment where the detrimental effects of homelessness are felt extremely heavily. In 2025 alone, 3,482 new cases of homelessness were reported in Newark, with about 174 estimated to be veterans. Bridges Outreach believes the true number is significantly higher, with the provided figure undercounting due to the difficulty of identifying this specific population (Zanchelli, 2025). With 350 people becoming homeless every month and few housing vouchers present in Newark, the veteran homeless crisis continues to worsen at a rate that existing intervention systems have proven unable to keep up with (Soucy & DuBois, 2025; Zanchelli, 2025).

Diagnosed mental disorders among newly homeless veterans increases to between 64 to 76% in the period immediately after losing housing, suggesting that untreated mental illness is not just a consequence of homelessness, but a direct cause (NCHV, 2024). Traumatic brain injury (TBI) diagnosis rates are two to three times higher in homeless veterans compared to housed veterans as well, further compounding the burden that this population carries on their mental health, and the presence of mental disorders has proven to be the strongest predictor of becoming homeless after military discharge (NCHV, 2024).

Homeless veterans are often surrounded by a society that overlooks these very dire issues. The transition from military to civilian life is already a major psychological challenge for a majority of veterans. Without adequate support systems upon their arrival, mental health conditions that could be relatively easily treated could go unaddressed for years, if not decades. By the time these veterans reach a state of chronic homelessness, the severity of their needs has likely grown significantly. The culture of stoicism and self-reliance often fostered during service further worsens the chance of a veteran identifying themselves as struggling and deciding to seek help (U.S. Department of Veterans Affairs, 2019). For Newark's homeless veteran population, this cultural barrier directly translates into delayed or entirely avoided treatment, allowing for

conditions that could have been addressed early to instead escalate the cycle of chronic homelessness. This results in a population of silent sufferers disconnected from valuable resources often until it is too late. The consequences of untreated mental illness among homeless veterans can be fatal. Suicide attempts and rate among this population far exceed those of both housed veterans and the general population and are directly linked to undiagnosed PTSD, isolation, and lack of support (Salaam, 2015). It is worth noting that veterans who served during the conflicts in Iraq and Afghanistan faced heightened suicide risk even when returning home, making the risk only greater if subject to homelessness at any point post-service (Kang et al., 2015). Compounding this crisis further, many homeless veterans turn to alcohol and drugs, using them as coping mechanisms in an attempt to avoid experiencing their everyday problems. This action, however, creates additional barriers to accessing VA services and housing stability (Salaam, 2015; U.S. Department of Veterans Affairs, 2019). Instead of addressing the issue, substance use ultimately results in further deterioration of veterans' mental health and lengthening the cycle of chronic homelessness. Research has further documented that substance abuse among homeless veterans often leads to legal complications, emergency hospitalizations, and permanent damage to personal relationships, expanding the timeline of chronic, potentially irreversible, homelessness (Salaam, 2015).

These interconnected crises create a public subject to intense isolation from those around them. Salaam (2015) documented total family estrangement among many homeless veterans caused directly by untreated mental illness and substance use, dividing them from oftentimes their last tie to society. As opportunities of reintegration fade, so do veterans' ability and confidence to seek help or maintain a sense of belonging with their surrounding environment (Marshall et al., 2020; Salaam, 2015). In Newark, homeless veterans constantly face these health consequences with little to no outside assistance, making the need for targeted intervention more urgent than ever (Zanchelli, 2025). Homelessness is not just a housing crisis, though that certainly is a big factor. It is instead a community-wide problem that affects every aspect of a veteran's social environment. Homeless veterans in Newark have seen various support systems, ranging from personal to institutional, break down. At the personal level, it is extremely common for these individuals to lose connection with their families, social networks, and stable housing. At the community level, homeless veterans face a particular stigma and distrust that further sever their connection with others and weaken their opportunities to reintegrate back into society safely and effectively. At the institutional level, also the broadest, systemic failures in attempts to care for homeless veterans have only continued the chronic cycle that many in Newark currently experience. Addressing veteran homelessness requires the use of interventions that understand and operate on all these levels to truly be successful (U.S. Department of Veterans Affairs, 2019). The depth and interconnectedness of these failures makes it clear that no single intervention can address the homeless veteran crisis alone. A coordinated, multi-layered process that meets veterans at each stage and in their own social environment simultaneously is required to successfully address the issue.

Research Rationale

The Housing First Program

Research has shown that the most commonly used interventions to address veteran homelessness have failed because they only focus on one dimension of the problem. A 2020 systematic review by Marshall et al. examined twenty-one studies on community integration interventions for homeless individuals and found that Housing First and other supportive housing programs, which are the most widely used approaches, produced mixed results or lacked effectiveness entirely when used on their own. Housing First, despite being the most widely funded and implemented veteran homelessness intervention in the United States, has consistently struggled to produce lasting outcomes when utilized without a complementary community-based support program. Veterans placed in housing without frequent care in other aspects of their lives, particularly their social network, frequently return to homelessness within months of being housed (Marshall et al., 2020). Employment-based programs also demonstrated limited effectiveness and little improvement in veterans' social support networks regardless of job placement (Marshall et al., 2020). This suggests that the barriers keeping homeless veterans from reintegrating into their communities are deeper than economic ones, instead being rooted in the psychological and social damage that homelessness causes over time.

Traditional Case Management Programs

Traditional case management, though logistically useful, has also shown a lack of effectiveness, as it has generally been unable to gain the trust of homeless individuals necessary in encouraging them to use available resources. This trust barrier is particularly apparent among veterans, with the President of Bridges Outreach, Richard Uniacke, stating that distrust of support systems is common in much of the general homeless population, it tends to be especially present among veterans and requires significantly more effort to overcome (Zanchelli, 2025). Further, Uniacke noted that outreach workers must navigate veterans' service histories, benefit eligibility, and mindsets of self-reliance. These factors are what make traditional case management models insufficient to this population without significant modification of the intervention (Zanchelli, 2025). A national survey of over 1,000 low-income veterans found that 35% of those experiencing homelessness did not know where to go for assistance, and 17.9% reported feeling too embarrassed to seek it (Marshall et al., 2020; Tsai & Kelton, 2022). Considering these numbers alongside the fact that 83% of homeless veterans who did successfully access care reported satisfaction with its quality and 75% reported satisfaction with its timeliness, it is apparent that the crisis is not a result of the quality of resources, but the gap between them and those who need them the most (Jones et al., 2021).

These barriers make it clear that effective intervention must be built around trust and lived experience as opposed to purely institutional systems. The continuous failure of these

approaches over multiple decades of implementation reflects the fundamental mismatch between the nature of the problem and the solution attempts being deployed.

Alternate models, such as Housing First, employment-based programming, and traditional case management were considered to be part of the NVRP, but ultimately excluded due to their documented limitations. These approaches address only one dimension of an extremely multi-layered problem and have consistently failed to produce lasting outcomes without extensive support (Marshall et al., 2020). The Newark Veteran Reintegration Program's three-model methodology was chosen due to the way the peer support specialist and harm reduction outreach models complement and reinforce each other, with the peer support building the trust necessary for veterans to engage with harm reduction services, while the harm reduction outreach allows for further relationship development between the veterans and peer support specialists.

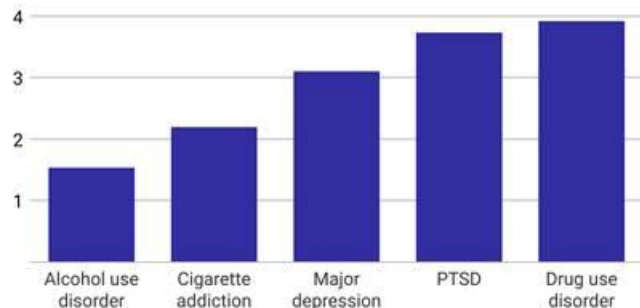
The Bridges Outreach Peer Support Program in Newark

Research points clearly toward a specific approach that clearly and continuously meets the qualifications of an effective intervention program: peer-led outreach programs. Psychosocial interventions involving peer support specialists, those with their own lived experiences of homelessness, have been documented to have the greatest impact on psychiatric improvement, days housed, and reduction of social isolation in homeless veterans (Marshall et al., 2020). This finding was consistent across multiple study designs and geographic contexts within the systematic review, making it one of the most supported conclusions in addressing veteran homelessness. When faced with someone who has been in similar—if not the same—circumstance as them, the feeling of embarrassment and the common stigma that keep many homeless veterans from seeking help is significantly reduced (Marshall et al., 2020). This allows them to make real connections and rebuild trust and social bonds that are vital for lasting reintegration into society. Bridges Outreach in Newark has already demonstrated this principle in practice, prioritizing the hiring of veterans with lived experience of homelessness onto their outreach teams. The organization's President, Richard Uniacke, credits this as a central component to Bridges Outreach's ability to build trust with the homeless veteran population (Zanchelli, 2025). Between 2020 and 2024, this approach focused on peer support contributed to a 31.2% reduction in unsheltered homelessness in Newark, providing substantial local evidence that the model works in the same environment that the Newark Veterans Reintegration Program is targeting (Soucy & DuBois, 2025).

Veterans with mental illness struggle to access care

Mental health and addiction issues are the biggest correlations of homelessness among vets, but getting help can carry a stigma.

5X more likely than other vets to suffer from



Data source: Cambridge University
By Elena Cox

stacker

Figure 1. Rates at which homeless veterans are more likely to suffer from mental health and addiction conditions compared to housed veterans (Cox, 2024).

The VA's Ending Veteran Homelessness Initiative, which incorporated coordinated community outreach and partnership in its methodology, produced meaningful national reductions in veteran homelessness when implemented (O'Toole et al., 2024). The most significant reductions occurred in communities where the initiative was implemented with the highest level of fidelity to its community partnership model. Coordination between organizations rather than a single intervention component was the greatest driver of success (O'Toole et al., 2024).

The decision to utilize the peer support specialist and in-person harm reduction outreach models as the major components of the NVRP came directly from the research findings. In twenty-one studies on community integration interventions for homeless individuals, peer support specialist interventions consistently produced the most positive results across all measured categories (Marshall et al., 2020). This includes psychiatric improvement, number of days housed, and reduction in social isolation, all of which outperformed the results of Housing First, employment-based programs, and traditional case management (Marshall et al., 2020). This method's consistency across various geographical locations and settings shows potential in working successfully in Newark's urban environment and was a major factor in its selection. The in-person harm reduction outreach model was selected based on the findings of a San Francisco-based pharmacist-led program in 2025. This proactive in-person delivery of harm reduction services increased veteran acceptance of these services from 10% to 36% (Rife-Pennington & Vu, 2025). This finding addresses the access barrier that has been found to be the primary obstacle between homeless veterans and available care. In addition, the San Francisco program found that many veterans report a deep mistrust of the healthcare system, further implying that pairing harm reduction outreach with peer support specialists, whose role is

to build trust with the veterans, makes for a methodology that accounts for every major access barrier.

The VA Harm Reduction Outreach Program in San Francisco

Proactive, community-based service delivery strengthens this model even further. Rife-Pennington and Vu (2025) present a pharmacist-led interdisciplinary street medicine program that brings harm-reduction services directly to homeless veterans in their own locale rather than relying on them to seek it out themselves. The San Francisco program's finding that in-person outreach increased veteran acceptance of harm reduction services by 10% to 36% demonstrates that the barrier is one of access, and that removing the burden that seeking care produces among homeless veterans will result in a dramatic improvement in engagement (Rife-Pennington & Vu, 2025). Programs of this archetype, though proven to be effective in breaking down access barriers and reaching the most isolated members of the target demographic, are dramatically underutilized. In Newark, where trust issues among homeless veterans are well-documented and housing availability is insufficient, bringing services directly to veterans rather than waiting for them to navigate complex institutional systems on their own is an absolutely necessary step to take (Zanchelli, 2025). The combination of peer-led outreach, proactive harm reduction delivery, and coordinated community support represents the clearest evidence-based model for the NVRP to adopt and deliver on its goal to address veteran homelessness in Newark.

The Bringing Veterans Home Initiative in New Jersey

A third model influencing the NVRP is the benefits and navigation framework established by New Jersey's Bringing Veterans Home Initiative. Research has identified awareness of and access to service as critical barriers to veteran service utilization. 35% of homeless veterans do not know where to go for help and 17.9% report feeling too embarrassed to seek it (Tsai & Kelton, 2022). The Bringing Veterans Home Initiative addresses this barrier directly by deploying case managers to identify eligible veterans and connect them to VA benefits, housing vouchers, and other state and federal resources. Despite being a new initiative, the BVHI has already received referrals from 254 veterans facing housing instability in Essex and Union County alone, demonstrating the scale of unmet need and the effectiveness of a proactive navigator-centered approach (Zanchelli, 2025). This model was selected for the Newark Veterans Reintegration Program because it directly addresses the access and awareness obstacle that many homeless veterans face and the peer outreach and harm reduction aspects of the program cannot fully resolve alone, with research showing that veterans who do successfully access care report overwhelmingly positive feedback with quality and timeliness (Jones et al., 2021).

The research behind the Newark Veterans Reintegration Program was conducted through a combination of peer-reviewed academic literature through the Rutgers University library

database, government agency publications, and reports from established organizations and state initiatives involved in addressing New Jersey's veteran homelessness crisis. Peer-reviewed sources were prioritized, including systematic reviews, phenomenological studies, and clinical program evaluations published in academic journals related to public health, social work, and pharmacy practice. The U.S. Department of Veterans Affairs and the U.S. Department of Housing and Urban Development provided data on the national scope of the risk factors and causes of veteran homelessness. Local and in-state data was sourced from the National Alliance to End Homelessness, Monarch Housing Associates, and reporting on New Jersey's Bringing Veterans Home Initiative, with in-depth focus on the specific conditions and demographics of the city of Newark and Essex County to avoid relying on national generalizations.

Plan

The Newark Veterans Reintegration Program

Newark, New Jersey has demonstrated the urgency of its homeless veteran crisis as well as the capacity to address it. Between 2020 and 2024, the city reduced unsheltered homelessness by 31.2% through community-driven strategies focused on street outreach and peer support. Despite this success, over 1,400 people still seek homeless services from the Essex/Newark Continuum every month and the subpopulation of veterans remains underserved (Soucy & DuBois, 2025). The Newark Veterans Reintegration Program is a twelve-month community-driven initiative designed to address the gap between homeless veterans and the services designed to help them. This will be done through coordinated peer-led outreach, proactive harm reduction services, benefits navigation, and community integration programming, with the program being built upon two existing models: the peer support specialist framework in Newark and the harm reduction outreach model in San Francisco. The former demonstrated success in building trust among homeless veterans and reducing the stigma they face through shared lived experience (Marshall et al., 2020). The latter brings healthcare and harm reduction services directly into veterans' own environment, breaking down the barriers that have likely prevented them from seeking assistance. The Newark Veterans Reintegration Program will be implemented in partnership with Bridges Outreach.

Peer Support Specialist Outreach

The most vital aspect of the NVRP is its peer support specialist outreach component, inspired by and modeled after the approach conducted by Bridges Outreach in Newark. Soucy and DuBois (2025) explain that Bridges Outreach prioritized hiring and working with veterans with lived experience of homelessness in their outreach teams because they can establish and build trust and common ground with currently homeless veterans in ways that traditional case managers cannot. Between 2020 and 2024, this approach resulted in a 31.2% reduction in

unsheltered homelessness, and the NVRP will continue building on that success. In the early stages of the program, four full-time peer support specialists will be deployed to conduct active street outreach throughout Newark, with more being deployed based on scale and outcomes. These workers will be responsible for identifying homeless veterans, building relationships with them, connecting them to VA benefits and other services, and supporting them throughout their housing placement process. Peer support specialist-based interventions have proven to produce the most consistent positive outcomes in psychiatric improvement, days housed, and reduced social isolation, with no other intervention alone coming close to having the same effects (Marshall et al., 2020). The greatest barrier keeping homeless veterans from receiving care is trust, and the peer component of this program is a simple and effective way to break down this wall. When speaking with someone they relate to, the stigma and embarrassment that prevent veterans from seeking help are greatly reduced, making lasting societal reintegration achievable.

In-Person Harm Reduction Outreach

The next major component of the NVRP is modeled after the pharmacist-led interdisciplinary program operating in San Francisco, California at a supportive housing facility. This program found that in-person visits with pharmacists increased veterans' acceptance of harm reduction services from 10% to 36%, demonstrating that proactive on-site outreach is more effective than institution-based contact (Rife-Pennington & Vu, 2025). A key finding from this effort is the fact that many veterans report a deep mistrust in the healthcare system, mirroring the barrier documented in the Newark homeless veteran population. Instead of expecting or hoping that veterans make the effort to navigate the complex institutional healthcare systems independently, the NVRP will deploy two clinical consultants to work with peer support specialists, delivering harm reduction services and providing access to medication, overdose prevention supplies, and substance use support directly to veterans. Given the Newark homeless veteran population's history of PTSD, depression, trauma, and substance abuse, incorporating proactive in-person harm reduction outreach is essential to the program's success (Salaam, 2015).

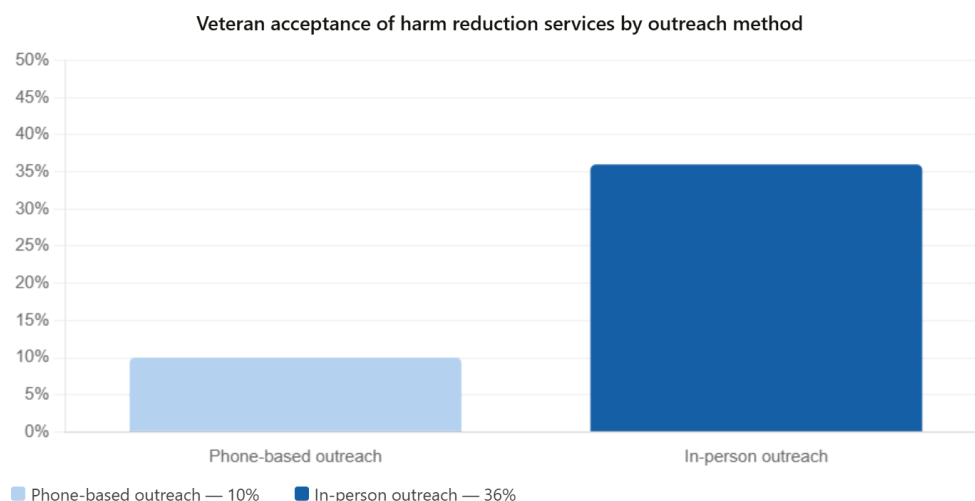


Figure 2. Veteran acceptance rates of harm reduction services comparing phone-based and in-person outreach methods at a San Francisco supportive housing facility (Rife-Pennington & Vu, 2025).

Benefits Navigation

An often-overlooked barrier that homeless veterans are faced with is the complexity of the systems which they must navigate to access the benefits intended for their use. With 35% of homeless veterans not knowing where to look for help and another 17.9% reporting feeling too embarrassed to seek it, resources often go unused even if they exist (Tsai & Kelton, 2022). The benefits navigation component of the NVRP is intended to fix this, drawing inspiration directly from the model established by New Jersey’s Bringing Veterans Home Initiative. This program, conducted throughout the state, deploys case managers to identify eligible veterans and connect them to VA benefits, housing vouchers, and other entitlements (Zanchelli, 2025). The initiative, though relatively new, has already received referrals from 254 veterans facing housing instability in Essex and Union County alone, demonstrating the scale of unmet need and the potential trust that veterans have in an intervention like this. Working alongside peer support specialists, dedicated benefits navigators deployed by the NVRP will help veterans identify and access the benefits they qualify for, including housing assistance, disability compensation, and VA healthcare enrollment. By providing assistance in navigating these complicated systems, a continuously-documented barrier will be meaningfully reduced, resulting in more veterans being able to achieve stability.

Community Integration Programming

The final component of the NVRP addresses what has been identified as the most lasting and psychologically damaging consequence of chronic homelessness in veterans. This is the erosion of social bonds, community, and sense of belonging that are necessary for effective recovery. Psychological interventions focused on rebuilding social networks and connections

within the community resulted in the most consistent long-term outcomes among homeless veterans, including improvements in psychiatric health, housing stability, and social integration (Marshall et al., 2020). By partnering with Bridges Outreach, the NVRP will establish community integration programming involving weekly group sessions, monthly veteran community events, and opportunities for veterans to build relationships with each other and the broader Newark community. This part of the program is modeled after the Community Support Program of New Jersey, which was documented to have successfully used drop-in wellness centers and peer-led group programming to rebuild social connections in Newark's general homeless population (Soucy & DuBois, 2025). Consistent and easily accessible opportunities for veterans to engage with others within the community will help revitalize the much-needed socialization that homelessness took away. Without a foundation of belonging and connection, housing placements and access to benefits are often insufficient in producing lasting change.

Stakeholder Roles

The successful implementation of the Newark Veterans Reintegration Program relies on the efforts of various key stakeholders. The Robert Wood Johnson Foundation acts as the primary funder, providing the financial resources needed to launch and maintain the program through its initial twelve-month period. Bridges Outreach, as NVRP's partner in the program's implementation, will contribute its existing infrastructure, community involvement, and outreach expertise to ensure the program's effectiveness within Newark's homelessness response system. The VA New Jersey Healthcare System will serve as a clinical support partner, helping to provide pathways and medical oversight for veterans in need of care beyond the scope of the NVRP's peer outreach and harm reduction components. Essex County and the city of Newark represent the broader scope in which the program operates, and their existing housing resources and social service networks are an important aspect of the benefits navigation component of the program. Lastly, Newark's homeless veteran population serves as the primary beneficiaries of this program and active participants in the peer support and community integration aspects of it. The engagement and feedback of these individuals are central to the program's continued success as well as its evaluation and refinement after twelve months.

Evaluation Plan

After the program's twelve-month introduction, the NVRP will undergo a formal evaluation. Three metrics will be the most useful to identify, those being veteran engagement and successful assistance, housing retention, and mental health outcomes. For veteran engagement, the program will track the total number of homeless veterans identified by peer support outreach specialists against the number who were successfully connected to at least one service, whether it is harm reduction services, benefits navigation, or community integration programming. A 50% or higher engagement rate will be considered a success, continuing to increase from the San Francisco project's jump from 10% to 36% acceptance of outreach (Rife-Pennington & Vu,

2025). Housing retention will be measured by tracking the number of veterans who maintain stable housing twelve months after their initial placement. Case manager continuity and consistent community support have been shown to be the strongest predictors of long-term housing retention, giving this metric a strong correlation with the program's community integration component (Soucy & DuBois, 2025). Mental health outcomes will be measured through self-reports done at the beginning and twelve-month mark of a veteran's process through the program, tracking changes in PTSD, depression, and social integration.

Results of the evaluation will be reported back to RWJF, documenting outcomes and recommendations for increasing the scale of the project further throughout New Jersey.

Budget

The estimated cost for the Newark Veterans Reintegration Program's first twelve months of operation is approximately \$503,000. This covers all personnel, harm reduction supplies, transportation, technology, community integration programming, and evaluation necessary to operate a functional program. The most effective interventions for veteran homelessness have proven to be those that utilize community outreach alongside trust-building service delivery, and the budget provided reflects the cost required to build a program like that in a city with the scale and diversity that Newark, New Jersey has.

<u>Item</u>	<u>Cost</u>
Personnel (4 peer support specialists, 1 program coordinator, 2 clinical consultants, 2 benefits navigators)	\$385,000
Harm reduction supplies	\$25,000
Transportation	\$10,000
Case management software, communication tools, and multilingual outreach materials	\$15,000
Community integration programming	\$12,000
Program evaluation	\$10,000
Administrative overhead	\$46,000
Total	\$503,000

Figure 3. Estimated twelve-month budget for the Newark Veterans Reintegration Program.

Discussion

Veteran Homelessness in Newark, New Jersey is an urgent yet solvable issue with the right efforts taken. By developing a trustworthy and comforting environment for this population while breaking down barriers that have kept them from receiving the help they need, this program will successfully take homeless veterans off the city streets and reintegrate them into society. These veterans will have opportunities to overcome the substance use, mental health, and social issues that reinforced continuous cycles of homelessness, allowing them to make meaningful connections and gain confidence in themselves and their lives. As such, this plan of action is a vital step to take to benefit the city of Newark and its many communities, first and foremost homeless veterans.

References

- Cox, E. (2024, April 30). Veteran homelessness is on the rise despite government efforts—here's how it happens. *Stacker*.
<https://stacker.com/stories/military/veteran-homelessness-rise-despite-government-efforts-heres-how-it-happens>
- Jones, A. L., Gordon, A. J., Gabrielian, S. E., Montgomery, A. E., Blosnich, J. R., Varley, A. L., deRussy, A. J., Austin, E. L., Hoge, A. E., Kim, Y., Gelberg, L., & Kertesz, S. G. (2021). Perceptions of Care Coordination Among Homeless Veterans Receiving Medical Care in the Veterans Health Administration and Community Care Settings. *Medical Care, Publish Ahead of Print*, 59(6). <https://doi.org/10.1097/mlr.0000000000001547>
- Kang, H. K., Bullman, T. A., Smolenski, D. J., Skopp, N. A., Gahm, G. A., & Reger, M. A. (2015). Suicide risk among 1.3 million veterans who were on active duty during the Iraq and Afghanistan wars. *Annals of Epidemiology*, 25(2), 96–100.
<https://doi.org/10.1016/j.annepidem.2014.11.020>
- Marshall, C. A., Boland, L., Westover, L. A., Marcellus, B., Weil, S., & Wickett, S. (2020). Effectiveness of interventions targeting community integration among individuals with lived experiences of homelessness: A systematic review. *Health & Social Care in the Community*, 28(6), 1843–1862. <https://doi.org/10.1111/hsc.13030>
- National Coalition for Homeless Veterans. (2024). *Veteran Homelessness*. National Coalition for Homeless Veterans. <https://nchv.org/veteran-homelessness/>

O'Toole, T. P., Pape, L. M., Kane, V., Diaz, M., Dunn, A., Rudolph, J. L., & Elnahal, S. (2024).

Changes in Homelessness Among US Veterans After Implementation of the Ending Veteran Homelessness Initiative. *JAMA Network Open*, 7(1), e2353778.

<https://doi.org/10.1001/jamanetworkopen.2023.53778>

Rife-Pennington, T. L., & Vu, T. T. (2025). Harm reduction services for veterans in supportive housing: A pharmacist-led, interdisciplinary, street medicine approach. *INNOVATIONS in Pharmacy*, 16(3). <https://doi.org/10.24926/iip.v16i3.6753>

Salaam, D. (2015). *A Phenomenological Study Examining the Lived Experience of Homeless Veterans in the Newark, New Jersey Metropolis*. Proquest.com.

https://www.proquest.com/citedreferences/MSTAR_2715512472/F719D725E31440B6PQ/1?accountid=13626&sourcetype=Dissertations%20&%20Theses

Soucy, D., & DuBois, N. (2025, August 26). Housing-focused responses to unsheltered homelessness: Spotlight on Newark, NJ. *National Alliance to End Homelessness*.

<https://endhomelessness.org/resources/case-studies/housing-focused-responses-to-unsheltered-homelessness/newark>

Tsai, J., & Kelton, K. (2022). Service use and barriers to care among homeless veterans: Results from the National Veteran Homeless and Other Poverty Experiences (NV-HOPE) study. *Journal of Community Psychology*, 51(1). <https://doi.org/10.1002/jcop.22912>

U.S. Department of Veterans Affairs. (2019). *Homelessness*. Va.gov.

<https://www.research.va.gov/topics/homelessness.cfm>

Zanchelli, N. (2025, November 13). *Many New Jersey Veterans Facing Hunger and Housing Instability*. TAPinto.

<https://www.tapinto.net/towns/newark/sections/essex-county-news/articles/many-new-jersey-veterans-facing-hunger-and-housing-instability>